1. **A description of the community health initiatives or programs that received funding from your hospital's 340B program participation, including how these investments are intended to improve access to care and quality of services. Specify the aggregate amount invested toward all such programs.**

   Gifford as a Health System

   Gifford's three organizational entities — Gifford Health Care, Gifford Medical Center, and Gifford Retirement Community — provide seamless, lifelong care from birth through end of life. The system includes a Critical Access Hospital, a network of Federally Qualified Health Centers (FQHC), and specialty services throughout the region. Gifford delivers comprehensive care with a 24-hour Emergency Department and inpatient unit, surgical services, an Adult Day Program, a 49-unit independent living facility, and a 30-bed nursing home. Gifford Health Care is the parent company for Gifford Medical Center and Gifford Retirement Community.

   Making Healthcare More Affordable for Vermonters

   Gifford Medical Center (GMC) benefited from an aggregate of $408,334.39 from its 340B program in FY2025. With the support of 340B funding, Gifford is able to extend the more favorable FQHC sliding scale to patients who receive hospital-based care. Maintaining consistency with financial support across our facilities makes the cost of care more predictable for our patients. In FY2025, 340B dollars covered nearly 40% of the over $1.1 million provided in financial assistance by GMC.

   Extending 340B Benefits to the Community Through Primary Care

   Gifford Health Care's FQHC also benefits from its own separate 340B program. The additional 340B funds enable Gifford to invest in services that reach underserved and high-risk community members. These services include school-based clinics in local elementary and high schools, expanded access to mental health counseling and psychiatric services, access to low-barrier substance use treatment, and more.

2. **A description of the hospital's internal review and oversight of its participation in the 340B program in compliance with the U.S. Department of Health and Human Services, Health Resources and Services Administration's 340B program rules and guidance.**

GMC performs monthly audits of 340B accumulations, provider eligibility, in-house drug purchases and usage. Each contract pharmacy's claims are also audited monthly. Quarterly reviews of OPAIS records, including the Medicaid Exclusion files and pricing reports are performed. Gifford hires an outside vendor to audit the 340B program annually. There are additional annual internal reviews of the entire 340B program, including child sites, contract pharmacies, TPAs, policies and procedures prior to HRSA/OPA mandated recertification. GMC maintains constant vigilance for HRSA program updates and subsequently updates internal policies and procedures to comply.

3. **A description of how the aggregated payment amount is calculated for payments made for all other internal expenses related to administering the 340B program during fiscal year 2025.**

   All 340B expenses are differentiated by an expense object code (EOC). Our 340B Program Coordinator reviews, approves and codes invoices to these 340B specified EOCs. Invoices are then processed through our Accounts Payable (AP) system. A fraction of the 340B Program Coordinator's salary and benefits are also coded to 340B EOCs in our payroll system. The AP and payroll system postings automatically update our General Ledger. Our 340B payments are calculated based on the expenses on our General Ledger.

   Regarding reimbursement for 340B drugs administered at the facility, we are not able to separately identify payments. We used an estimate based on our ratio of reimbursement to charge by payer for the place of service that the drugs were provided. As a critical access hospital, any reimbursement received for services is an estimate until the Medicare cost report is filed. Critical Access Hospitals are ultimately paid cost by Medicare and any fee for service revenue is a placeholder until the cost report is filed.